

1. A previously healthy 39-year-old woman at 37 weeks' gestation comes to the emergency department 2 hours after the onset of acute pain in the left hemithorax. She says that the pain is exacerbated by breathing. Her temperature is 38.2°C (100.8°F), pulse is 120/min, respirations are 24/min, and blood pressure is 110/70 mm Hg. Fetal heart tones are 170/min. Examination shows no abnormalities. An x-ray of the chest shows no abnormalities. An ECG shows nonspecific changes. Arterial blood gas analysis on room air shows:

pH	7.43
Pco ₂	35 mm Hg
Po ₂	70 mm Hg

Which of the following is the most likely diagnosis?

- ☐ A) Angina pectoris
- ☐ B) Costochondritis
- ☐ C) Myocardial infarction
- ☐ D) Pulmonary embolus
- ☐ E) Viral pneumonia

2. A 52-year-old woman comes to the physician for a routine examination. She has mild hypothyroidism well controlled with levothyroxine. Five years ago, she was diagnosed with stage I breast cancer; she completed tamoxifen therapy 1 year ago and is currently in remission. Menopause occurred 4 years ago. Her mother sustained a femoral fracture in a bicycle collision at the age of 55 years. The patient has smoked two cigarettes weekly for 25 years. She is 150 cm (4 ft 11 in) tall and weighs 43 kg (95 lb); BMI is 19 kg/m². Examination shows no thyromegaly. There is a small, well-healed surgical scar over the upper outer quadrant of the left breast. The remainder of the breast and pelvic examinations show no abnormalities. Which of the following historical findings is the greatest risk factor for osteoporotic fracture in this patient?

- ☐ A) BMI
- ☐ B) Family history of fracture
- ☐ C) Levothyroxine therapy
- ☐ D) Tamoxifen therapy
- ☐ E) Tobacco use



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

3. A 20-year-old woman comes to the physician because of a 1-week history of vaginal discharge. She is sexually active with one partner, and they use condoms inconsistently. She reports that he was recently treated for syphilis. Physical examination of the patient shows no abnormalities. Pelvic examination shows white verrucous lesions over the upper vaginal wall and cervix. A Pap smear is reported as atypical squamous cells. Human papillomavirus testing is negative for high-risk types. Which of the following is the most likely diagnosis?

- ☐ A) Bacterial vaginosis
- ☐ B) Cervical intraepithelial neoplasia (CIN) 2
- ☐ C) Condylomata acuminata
- ☐ D) Herpes simplex
- ☐ E) Secondary syphilis



4. A 25-year-old woman comes to the physician because of a 4-week history of right-sided pelvic pain. She has been sexually active with one partner for 3 years; they use condoms for contraception. Her last menstrual period was 3 weeks ago. She is afebrile. Pelvic examination shows a 12-cm, cystic, mobile, right adnexal mass. Her serum CA 125 concentration is 35 U/mL (N<35). Serum β -hCG testing is negative. An x-ray of the abdomen shows calcium deposits in the mass. Which of the following is the most likely diagnosis?

- ☐ A) Cystic teratoma
- ☐ B) Ectopic pregnancy
- ☐ C) Fallopian tube carcinoma
- ☐ D) Follicular cyst
- ☐ E) Leiomyomata uteri
- ☐ F) Ovarian carcinoma



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

5. Five weeks after the uncomplicated delivery of her newborn, a 25-year-old woman, gravida 1, para 1, comes to the physician with concerns about insomnia, restlessness, extreme fatigue, irritability, and depression. She has not been eating well. She finds the baby's crying increasingly annoying and has been letting him cry alone in the crib for long periods of time. She is having difficulty with her memory and ability to concentrate. While her husband has been supportive, she has had difficulty involving him in the care of the baby. She is afraid that she may hurt her child and says that maybe she "would be better off dead." Which of the following is the most appropriate initial step in management?
- ☐ A) Reassure the patient that her postpartum "blues" are normal
 - ☐ B) Measure serum estradiol concentration
 - ☐ C) Prescribe immediate antidepressant therapy
 - ☐ D) Refer the patient to a family therapist
 - ☐ E) Arrange for an immediate psychiatric evaluation



6. A 36-year-old woman, gravida 1, para 1, has been unable to conceive for 1 year. Her vaginal delivery 2 years ago was complicated by postpartum hemorrhage and endometritis treated with dilatation and curettage. She has not resumed menses since delivery but does have cyclic abdominal pain. Examination shows normal findings. Her husband's semen analysis shows normal findings. Which of the following is the most likely cause?

- ☐ A) Anovulation
- ☐ B) Antisperm antibodies
- ☐ C) Endometriosis
- ☐ D) Luteal phase defect
- ☐ E) Periadnexal adhesions
- ☐ F) Tubal occlusion
- ☐ G) Uterine synechiae



7. A 32-year-old woman, gravida 2, para 1, at 27 weeks' gestation is brought to the emergency department because of a 3-hour history of painful contractions every 5 minutes. She has not had vaginal discharge or itching. During her first prenatal visit, routine urine culture grew 10,000 colonies/mL of *Streptococcus agalactiae* (group B). She was not treated at that time because she was asymptomatic. She has no history of serious illness. Pregnancy had been otherwise uncomplicated. She takes no medications and has an allergy to penicillin. Her first pregnancy ended in spontaneous vaginal delivery at 33 weeks' gestation. Her temperature is 37.8°C (100°F), pulse is 100/min, and blood pressure is 90/50 mm Hg. Abdominal examination shows tenderness to percussion over the right flank. The uterus is nontender and consistent in size with a 27-week gestation. The fetal heart rate is 160/min. Uterine contractions occur every 5 minutes and last 45 seconds. On pelvic examination, the cervix is 2 cm dilated and 75% effaced; the vertex is at 0 station. Which of the following is the most likely cause of this patient's symptoms?

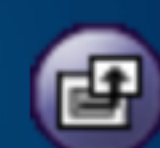
- ☐ A) Chorioamnionitis
- ☐ B) Cervical insufficiency
- ☐ C) Premature rupture of membranes
- ☐ D) Pyelonephritis
- ☐ E) Sepsis



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

8. A 22-year-old woman, gravida 2, para 1, at 38 weeks' gestation comes to the physician for a routine prenatal visit. She feels well other than intermittent, mild, low back pain, and her pregnancy has been uncomplicated. Her pulse is 100/min, and blood pressure is 130/90 mm Hg. Physical examination shows no abnormalities. The estimated fetal weight is 3629 g (8 lb). The fetal heart rate is 120/min. The cervix is 4 cm dilated and 100% effaced; the vertex is at –1 station. Which of the following is the most likely cause of these findings?

- ☐ A) Braxton-Hicks contractions
- ☐ B) Cervical insufficiency
- ☐ C) Gestational hypertension
- ☐ D) Premature rupture of membranes
- ☐ E) Normal labor



9. A 15-year-old girl is brought to the physician because of a 3-day history of fever, abdominal pain, and nausea. She also has had a thick, white vaginal discharge. Menarche was at the age of 13 years, and her menses occur at irregular 28- to 40-day intervals. She became sexually active 1 month ago and uses condoms inconsistently. Her temperature is 39.6°C (103.2°F), pulse is 108/min, respirations are 20/min, and blood pressure is 102/80 mm Hg. Examination shows lower abdominal tenderness. Pelvic examination shows pain with cervical motion and adnexal tenderness with a 3-cm mass. A Gram stain of vaginal discharge shows gram-negative diplococci. Which of the following is the most likely explanation for this patient's susceptibility to this condition?

- ☐ A) Immature immune system
- ☐ B) Increased cervical cell vulnerability to infections
- ☐ C) Increased concentrations of progesterone
- ☐ D) Latex hypersensitivity
- ☐ E) Menarche before 14 years of age



10. A previously healthy 5-year-old girl is brought to the physician because of a 2-day history of a foul-smelling, yellow discharge and vaginal spotting on her underpants. She has not had pain with urination. Physical examination shows no abnormalities. Pelvic examination shows mild vulvar erythema and a purulent discharge at the vaginal opening. Which of the following is the most likely cause?

- ☐ A) Allergic reaction to soaps
- ☐ B) Candidal vaginitis
- ☐ C) Rhabdomyosarcoma
- ☐ D) Sexual abuse
- ☐ E) Vaginal foreign body



11. A moderately obese 27-year-old woman, gravida 1, para 1, comes to the physician because of pain and tenderness in her left thigh for 2 days. She delivered a healthy newborn at term 6 days ago. Examination shows tenderness and swelling over the left thigh and calf. There is pain in the left calf with dorsiflexion of the left foot. Which of the following is the most appropriate next step to confirm the diagnosis?

- ☐ A) Doppler flow study
- ☐ B) Duplex venous ultrasonography
- ☐ C) Impedance plethysmography
- ☐ D) Fibrinogen leg scan
- ☐ E) Ventilation-perfusion lung scans



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

12. A 21-year-old primigravid woman at 8 weeks' gestation comes to the physician for her first prenatal visit. She has sickle cell disease and is concerned about the risk for transmitting this disease to her fetus. Her pregnancy has been otherwise uncomplicated. Examination shows a uterus consistent in size with an 8-week gestation. Her husband's hemoglobin electrophoresis shows:

Hemoglobin A	42% (N=95%–98%)
Hemoglobin A ₂	3% (N=2%–3%)
Hemoglobin F	2% (N=0.8%–2%)
Hemoglobin S	53% (N=0%)

This patient should be counseled that the probability her fetus will have sickle cell disease is closest to which of the following?

- ☐ A) 25%
- ☐ B) 40%
- ☐ C) 50%
- ☐ D) 100%
- ☐ E) No risk

13. A 37-year-old primigravid woman at 35 weeks' gestation comes to the physician for a routine prenatal visit. She has been receiving routine prenatal care since 8 weeks' gestation. Pregnancy has been complicated by the onset of hypertension at 18 weeks' gestation that has been well controlled with labetalol. She has an 18-year history of type 1 diabetes mellitus. Her hemoglobin A_{1c} at 8 weeks' gestation was 5.7%. Her blood pressure now is 140/90 mm Hg. Examination shows a soft uterus with a fundal height of 32 cm. The fetal heart rate is 140/min. Serum studies show a urea nitrogen concentration of 8 mg/dL and creatinine concentration of 1 mg/dL. A 24-hour urine collection shows a protein concentration of 800 mg. Ultrasonography shows a normal fetus at the 5th percentile for weight; the placenta is anterior and fundal. There is a mild decrease in the amount of amniotic fluid. Which of the following is the most likely cause of the ultrasound findings?

- ☐ A) Adverse drug effect
- ☐ B) Fetal congenital malformation
- ☐ C) Incorrect gestational age
- ☐ D) Placental dysfunction
- ☐ E) Trisomy 21



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

14. A 57-year-old woman, gravida 3, para 3, comes to the physician because of loss of urine for 2 weeks. She now wears an absorbent pad constantly and frequently soaks the pad. All of her children were delivered vaginally with no complications. Three years ago, she received radiation therapy for cervical cancer and has been cancer free since then. She has no other history of serious illness and takes no medications. Vital signs are within normal limits. Physical examination shows no abnormalities. Genitourinary examination shows thin, atrophic, moist vaginal mucosa; there is a pool of fluid in the vaginal canal. Her postvoid residual volume is less than 10 mL. Urinalysis shows no abnormalities. Which of the following is the most likely diagnosis?

- ☐ A) Overactive bladder
- ☐ B) Overflow incontinence
- ☐ C) Stress incontinence
- ☐ D) Urinary retention
- ☐ E) Urinary tract infection
- ☐ F) Vesicovaginal fistula



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

15. A 57-year-old woman, gravida 6, para 6, comes to the physician because of a 6-month history of immediate loss of urine when she coughs or exercises. Her children were born by vaginal delivery. Her last menstrual period was 10 years ago, and she has never received estrogen therapy. Pelvic examination shows a normal-sized uterus and ovaries. There is downward mobility of the urethral vesical junction with Valsalva maneuver. Which of the following is the most likely diagnosis?

- ☐ A) Cystocele
- ☐ B) Detrusor instability
- ☐ C) Stress incontinence
- ☐ D) Urethral prolapse
- ☐ E) Urethrocele
- ☐ F) Uterovaginal prolapse



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

16. A 15-year-old girl is brought to the physician because of a 1-week history of constant severe abdominal pain. During the past year, she has had 10 episodes of cramps; each episode lasts 3 to 5 days. She has no history of serious illness and takes no medications. She has never had a menstrual period. She is sexually active with one male partner, and they do not use contraception. She is at the 80th percentile for height and weight. Her temperature is 37.8°C (100°F), pulse is 120/min, respirations are 16/min, and blood pressure is 90/50 mm Hg. Breast and genital development is Tanner stage 5. Abdominal examination shows moderate tenderness. A mass is palpated in the suprapubic region at the midline. Pelvic examination shows normal-appearing external genitalia and lower vagina. The cervix cannot be visualized because of bluish bulging vaginal tissue that obscures the upper vagina. Which of the following is the most appropriate next step in management?

- ☐ A) Gonadotropin-releasing hormone agonist therapy
- ☐ B) Nonsteroidal anti-inflammatory drug therapy
- ☐ C) Oral contraceptive therapy
- ☐ D) Transvaginal incision and drainage
- ☐ E) Laparotomy



17. A 27-year-old woman who is a long-distance runner comes to the physician because of a 1-month history of postcoital spotting lasting 1 to 2 days. Menses have occurred at regular intervals since menarche at the age of 14 years. She underwent a bilateral tubal ligation 9 months ago. She is sexually active with a new partner. Which of the following is the most likely diagnosis?

- ☐ A) Blood dyscrasia
- ☐ B) Broad-ligament leiomyomata uteri
- ☐ C) Cervicitis
- ☐ D) Exercise-related menstrual irregularity
- ☐ E) Ovarian cancer
- ☐ F) Polycystic ovarian syndrome
- ☐ G) Pregnancy
- ☐ H) Premature ovarian failure



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

18. A 32-year-old woman comes to the physician because of a 3-year history of increasingly irregular menses. Menses had occurred at irregular 45- to 60-day intervals since menarche at the age of 14 years, but now her cycle interval is up to 90 days. Her menstrual periods typically last 2 to 7 days. She is otherwise asymptomatic and has no history of serious illness. She is 163 cm (5 ft 4 in) tall and weighs 103 kg (227 lb); BMI is 39 kg/m². Her pulse is 80/min, and blood pressure is 140/80 mm Hg. Physical examination shows velvety pigmented skin over the axillae and groin. Which of the following serum hormone concentrations is most likely to be increased in this patient?

- ☐ A) Cortisol
- ☐ B) Follicle-stimulating hormone
- ☐ C) Progesterone
- ☐ D) Testosterone
- ☐ E) Thyroid-stimulating hormone



19. A 27-year-old woman, gravida 2, para 1, aborta 1, comes to the physician because of a 3-day history of increasing abdominal pain and a 1-day history of fever. Five days ago, she underwent an uncomplicated abortion at 10 weeks' gestation and received prophylactic antibiotics. Her temperature is 39.2°C (102.6°F), pulse is 110/min, respirations are 24/min, and blood pressure is 90/50 mm Hg. Physical examination shows a tender uterus consistent in size with a 12-week gestation and no adnexal masses. Pelvic ultrasonography shows a heterogeneous endometrial mass. After intravenous antibiotic therapy is started, she undergoes suction dilatation and curettage followed by sharp curettage to remove the infected material. This patient is at increased risk for which of the following conditions?

- ☐ A) Amenorrhea
- ☐ B) Endometrial cancer
- ☐ C) Leiomyomata uteri
- ☐ D) Ovarian cancer
- ☐ E) Polycystic ovarian syndrome
- ☐ F) Premature ovarian failure



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

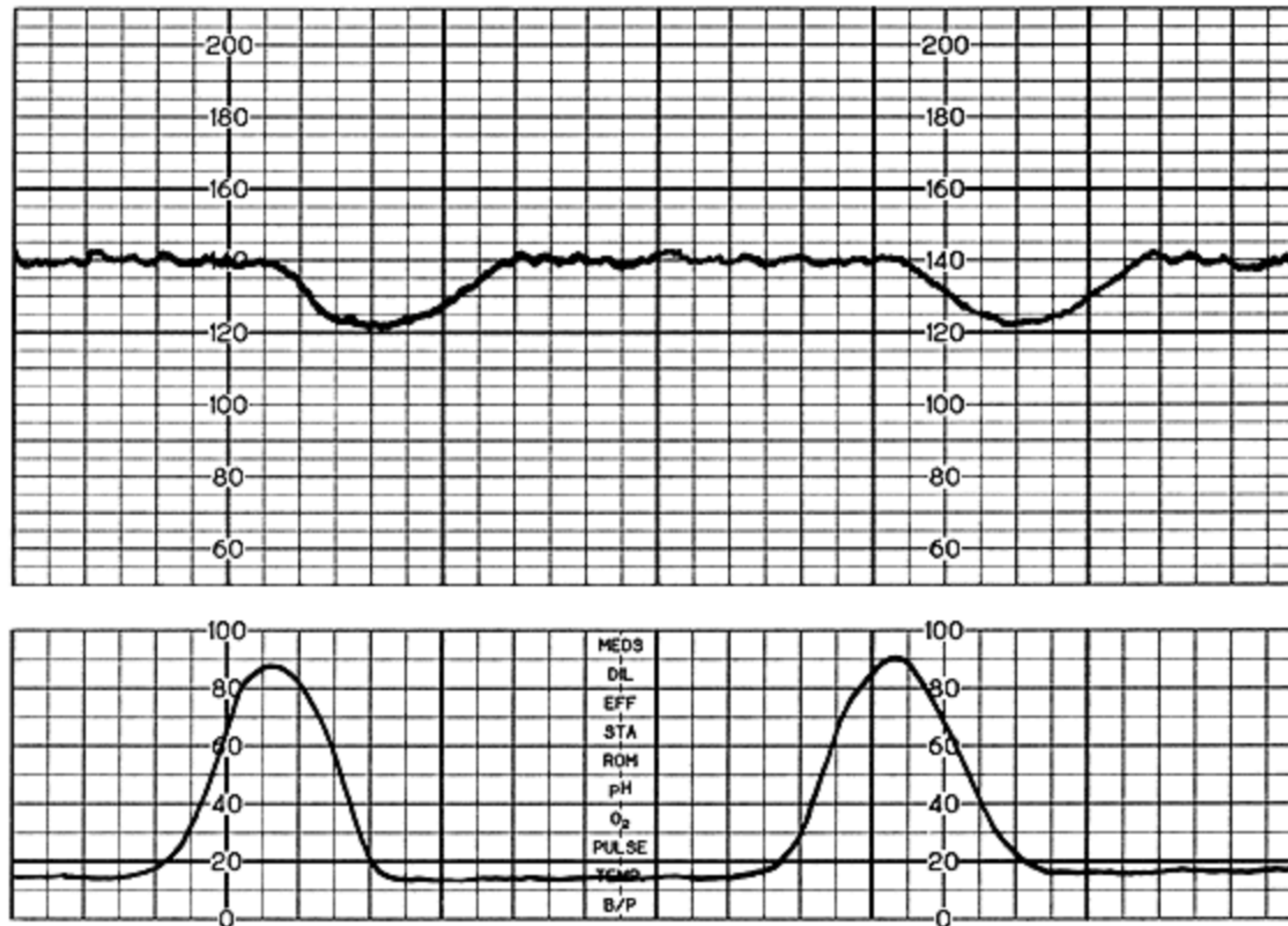
20. A 19-year-old woman comes to the emergency department because of moderate lower abdominal pain and vaginal spotting that began after her last menstrual period 2 weeks ago. Menses occur at regular 28-day intervals. She underwent a first trimester elective abortion 8 months ago and has been using an oral contraceptive since then. She has been sexually active with one male partner for 1 year. Her temperature is 37.6°C (99.6°F); other vital signs are within normal limits. Abdominal examination shows no tenderness. Pelvic examination shows blood-tinged discharge at the cervical os. There is cervical motion and mild uterine tenderness. A urine pregnancy test is negative. Which of the following is the most likely cause of this patient's symptoms?
- ☐ A) *Chlamydia trachomatis* infection
 - ☐ B) Endometrial polyp
 - ☐ C) Levonorgestrel-induced endometrial atrophy
 - ☐ D) Retained products of conception
 - ☐ E) Trichomoniasis



21. A 17-year-old primigravid patient comes to the physician because she has had decreased fetal movement over the past 2 days. She does not recall the date of her last menstrual period and has had only one previous prenatal visit. Examination shows a uterus consistent in size with a 32-week gestation. Ultrasonography shows a biparietal diameter consistent with a 31-week gestation. There is a duodenal bubble and flaccid tone of the fetus. Which of the following is the most likely cause of these findings?

- ☐ A) Congenital megacolon (Hirschsprung disease)
- ☐ B) Down syndrome
- ☐ C) Fetal growth restriction
- ☐ D) Gonadal dysgenesis 45,X (Turner syndrome)
- ☐ E) Placental insufficiency



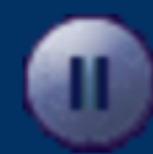
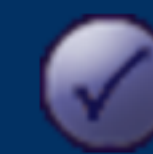


22. A 34-year-old woman, gravida 4, para 3, at 40 weeks' gestation is admitted because of a blood pressure of 160/95 mm Hg. She has chronic hypertension and is being treated with methyldopa. Ultrasonography shows fetal growth restriction. Four hours after induction of labor with oxytocin, the cervix is 6 cm dilated. A fetal heart tracing is shown. Which of the following is the most likely explanation for the findings shown in the fetal heart tracing?

- ☐ A) Compensated fetal stress
- ☐ B) Umbilical cord compression
- ☐ C) Pressure on the fetal head
- ☐ D) Uteroplacental insufficiency
- ☐ E) Normal fetal heart rate

23. A 22-year-old woman comes to the physician because of a 2-week history of increasingly severe vaginal burning and discharge. She has been sexually active with one partner for 1 year, and they use condoms consistently for contraception. Examination shows normal external genitalia and a gray frothy vaginal discharge. The pH of the vaginal discharge is 5; microscopic examination shows squamous epithelial cells coated with bacteria. Which of the following is the most appropriate pharmacotherapy?

- ☐ A) Hydrogen peroxide douches
- ☐ B) Oral azithromycin
- ☐ C) Vaginal estrogen
- ☐ D) Vaginal metronidazole gel
- ☐ E) Vaginal miconazole



24. A 23-year-old primigravid woman at 33 weeks' gestation is admitted to the hospital because she has not felt fetal movement for 2 days. Pregnancy had been uncomplicated. She has no history of serious illness. Her sister has had three spontaneous abortions. The patient's temperature is 37°C (98.6°F), pulse is 80/min, and blood pressure is 110/60 mm Hg. Examination shows a soft uterus consistent in size with a 33-week gestation. Fundal height is 34 cm. Fetal heart tones cannot be auscultated. Ultrasonography confirms intrauterine fetal demise. Laboratory studies show the patient to be a homozygote for the factor V Leiden mutation. Thrombosis of which of the following is the most likely cause of the fetal demise?

- ☐ A) Fetal cerebral arteries
- ☐ B) Fetal ductus arteriosus
- ☐ C) Maternal lower extremity
- ☐ D) Umbilical cord
- ☐ E) Uteroplacental artery



25. An 18-year-old primigravid woman at 39 weeks' gestation delivers a newborn 2 days after developing chickenpox. The pregnancy had been otherwise uncomplicated. Which of the following is the most appropriate care for the newborn?

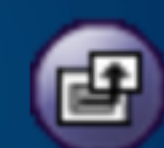
- ☐ A) Observation for signs of infection
- ☐ B) Acyclovir therapy
- ☐ C) Immune globulin therapy
- ☐ D) Interferon therapy
- ☐ E) Varicella-zoster immune globulin therapy



Previous



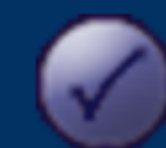
Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

26. A widowed 37-year-old woman, gravida 3, para 3, whose youngest child is 10 years old, has had increasingly heavy but regular menstrual periods for the past 3 years. Her last menses occurred 2½ weeks ago. She reports a 2-day history of labor-like pains accompanied by a small amount of vaginal bleeding. Speculum examination shows that the cervix is moderately effaced and 2 cm dilated, with some beefy red tissue at the os. Which of the following is the most likely diagnosis?

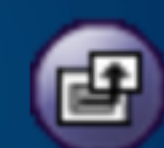
- ☐ A) Carcinoma of the cervix
- ☐ B) Carcinoma of the endometrium
- ☐ C) Incomplete abortion
- ☐ D) Pedunculated submucous leiomyoma uteri
- ☐ E) Sarcoma of the uterus



Previous



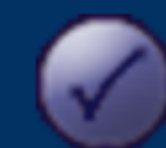
Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

27. A 27-year-old primigravid woman at 7 weeks' gestation comes to the physician for her first prenatal visit. She has no history of serious illness, and pregnancy has been uncomplicated. She began taking a prenatal vitamin 3 weeks ago. Physical examination, including pelvic examination, shows no abnormalities. Laboratory studies show:

Hemoglobin	10.1 g/dL
Hematocrit	30%
Mean corpuscular volume	72 μm^3
Serum	
Ferritin	110 ng/mL
Iron	130 $\mu\text{g/dL}$

Which of the following is the most appropriate next step in diagnosis?

- ☐ A) α -Globin DNA testing
- ☐ B) Hemoglobin electrophoresis
- ☐ C) Hemoglobin solubility testing
- ☐ D) Measurement of serum folic acid concentration
- ☐ E) Measurement of serum vitamin B₁₂ (cobalamin) concentration

28. A 42-year-old woman, gravida 2, para 1, at 20 weeks' gestation comes to the physician for a routine prenatal visit. She has tested positive for the factor V Leiden mutation. Her first pregnancy was uncomplicated, and she delivered at term by cesarean section for breech presentation. She has smoked one pack of cigarettes daily for 20 years and has continued to smoke during pregnancy. She does not drink alcohol. She maintains a vegan diet, and she owns and manages a vegetarian restaurant. She lives at home with her partner and son. Her temperature is 37°C (98.6°F), pulse is 80/min, respirations are 16/min, and blood pressure is 100/60 mm Hg. Physical examination shows no abnormalities. Ultrasonography shows a fetus consistent in size with a 20-week gestation with normal amniotic fluid volume. The placenta is anterior, and the distal portion covers the internal cervical os. Which of the following is the greatest predisposing factor for this patient's findings on ultrasonography?

- ☐ A) Maternal age
- ☐ B) Nutritional status
- ☐ C) Previous cesarean delivery
- ☐ D) Smoking history
- ☐ E) Thrombophilia



29. A 36-year-old woman, gravida 2, para 1, at 41 weeks' gestation has had ruptured membranes without contractions for 8 hours. Her first infant weighed 4422 g (9 lb 12 oz) at birth. This pregnancy has been uncomplicated except for gestational diabetes, which was diagnosed at 26 weeks' gestation and has been well controlled with diet. Initial assessment shows a fundal height of 40 cm. On ultrasonography, the estimated fetal weight is 3714 g (8 lb 3 oz). The cervix is 2 cm dilated and 50% effaced. The fetal heart rate is within normal limits. Labor is induced with intravenous oxytocin. Four hours later, her cervix is 4 cm dilated and completely effaced. Continuous epidural anesthesia is administered. Two hours later, the fetal heart rate pattern demonstrates late decelerations with each contraction. The contractions occur every minute, last 45 seconds, and are 75 mm Hg at their peak. Which of the following is the most likely explanation for this pattern?

- ☐ A) Epidural anesthesia
- ☐ B) Fetal macrosomia
- ☐ C) Gestational diabetes
- ☐ D) Oxytocin administration
- ☐ E) Postdates pregnancy



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

30. A 42-year-old woman comes to the physician because of a 3-month history of urinary urgency and frequency and occasional incontinence and a 2-month history of numbness below her waist. She has had to get up at least three times at night to void. She has not had fever, pain with urination, or cloudy urine. She has relapsing-remitting multiple sclerosis; symptoms have included double vision 10 years ago and right leg weakness 3 years ago. Sensation to vibration is decreased over both feet. Deep tendon reflexes are 3+ at the knees. Urinalysis is within normal limits. Her postvoid residual volume is 45 mL. Which of the following is the most likely cause of this patient's urinary findings?

- ☐ A) Detrusor hyperreflexia
- ☐ B) Detrusor hypotonia
- ☐ C) External sphincter hyperactivity
- ☐ D) External sphincter incompetence
- ☐ E) Overflow incontinence secondary to bladder outlet obstruction



31. A 32-year-old primigravid woman at term has a cervix that has remained 5 cm dilated over the past 4 hours despite the administration of oxytocin. Contractions occur every 3 minutes and are 55 to 64 mm Hg by intrauterine pressure catheter measurement. Examination shows a somewhat molded vertex and considerable caput succedaneum. Which of the following is the most likely diagnosis?

- ☐ A) Arrest of active phase
- ☐ B) Hypotonic contractions
- ☐ C) Protracted latent phase
- ☐ D) Normal active phase
- ☐ E) Normal second stage



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

32. A 32-year-old woman, gravida 5, para 4, at 40 weeks' gestation is brought to the emergency department because of bright red vaginal bleeding for 1 hour. Contractions have occurred every 2 to 3 minutes for 5 hours. Her pregnancy had been uncomplicated. Ultrasonography at 22 weeks' gestation showed no evidence of placenta previa. Her temperature is 37°C (98.6°F), pulse is 120/min, respirations are 18/min, and blood pressure is 90/60 mm Hg. Physical examination shows a tender abdomen with a firm uterus. The fetal heart rate is 140/min. On speculum examination, there is copious bright red bleeding from the external cervical os. The cervix is 4 cm dilated and 80% effaced. The physician informs the patient and her husband that emergency cesarean delivery is recommended. The patient and her husband state that, because of their religious beliefs, the elders of their church must agree to a surgical delivery before they can give their consent. Which of the following is the most appropriate course of action?
- ☐ A) Ask the patient to convene a meeting of the church elders to discuss cesarean delivery
 - ☐ B) Transfer the patient to another physician
 - ☐ C) Appoint a guardian ad litem for the fetus to consent to cesarean delivery
 - ☐ D) Obtain approval from the hospital administrator to perform emergency cesarean delivery
 - ☐ E) Obtain approval from the pediatric staff to perform emergency cesarean delivery



33. A 25-year-old primigravid woman is admitted in labor at 39 weeks' gestation. The cervix is 6 cm dilated and 100% effaced. The presenting part is not palpable. Fetal heart rate is 140/min. The estimated fetal weight is 3200 g (7 lb 1 oz). Which of the following is the most appropriate next step in management?

- ☐ A) X-ray pelvimetry
- ☐ B) Ultrasonography
- ☐ C) Oxytocin augmentation
- ☐ D) Amniotomy
- ☐ E) Cesarean delivery



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

34. An asymptomatic 52-year-old nulligravid woman comes to the physician for a routine health maintenance examination. Her last visit to a physician was 4 years ago. Menses occur at regular 30- to 32-day intervals; there is no bleeding between menses. She smoked one pack of cigarettes daily but quit 5 years ago; she drinks one to two glasses of wine per week. In addition to a Pap smear, mammography, and cholesterol testing, which of the following tests is most appropriate at this time?

- ☐ A) X-rays of the chest
- ☐ B) Pelvic ultrasonography
- ☐ C) Sputum cytology
- ☐ D) Bone densitometry
- ☐ E) Colonoscopy



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

35. A 42-year-old woman comes to the physician because of a 1-year history of vaginal bleeding for 2 to 5 days every 2 weeks. The flow varies from light to heavy with passage of clots. Menses previously occurred at regular 25- to 29-day intervals and lasted for 7 days with heavy flow. She has no history of serious illness and takes no medications. She is sexually active with one male partner, and they use condoms inconsistently. Her mother died of colon cancer, and her maternal grandmother died of breast cancer. She is 163 cm (5 ft 4 in) tall and weighs 77 kg (170 lb); BMI is 29 kg/m². Her temperature is 36.6°C (97.8°F), pulse is 90/min, respirations are 12/min, and blood pressure is 100/60 mm Hg. The uterus is normal sized. The ovaries cannot be palpated. The remainder of the examination shows no abnormalities. Test of the stool for occult blood is negative. An endometrial biopsy specimen shows grade I adenocarcinoma with atypical endometrial hyperplasia. Which of the following is the most appropriate next step in management?

- ☐ A) Medroxyprogesterone therapy
- ☐ B) Oral contraceptive therapy
- ☐ C) Endometrial ablation
- ☐ D) Dilatation and curettage
- ☐ E) Hysterectomy

36. A 30-year-old woman, gravida 3, para 2, at 10 weeks' gestation has had fever, minimal vaginal bleeding, and severe pelvic pain for 36 hours. She says that this is an unintended pregnancy. Four months ago, she was treated with oral antibiotics for a pelvic infection. Her temperature is 40°C (104°F). Examination shows a foul-smelling vaginal discharge and a small laceration on the cervix. The uterus is extremely tender and consistent in size with a 10-week gestation. Leukocyte count is 18,000/mm³ with a left shift. Which of the following is the most likely diagnosis?

- | | |
|--|--|
| ☐ A) Acute appendicitis | ☐ H) Leiomyomata uteri |
| ☐ B) Corpus luteum cyst | ☐ I) Multiple gestation |
| ☐ C) Ectopic pregnancy | ☐ J) Pelvic inflammatory disease |
| ☐ D) Fetal congenital abnormalities | ☐ K) Septic abortion |
| ☐ E) Gestational trophoblastic disease | ☐ L) Single gestation |
| ☐ F) Hyperemesis gravidarum | ☐ M) Threatened abortion |
| ☐ G) Inevitable abortion | ☐ N) Torsion of the adnexa |



37. A 32-year-old primigravid woman at 10 weeks' gestation comes for her first prenatal visit. She has a history of two to three seizures yearly, but she has had two seizures over the past 2 weeks. Medications include phenytoin. Examination shows a uterus consistent in size with a 10-week gestation. Her serum phenytoin concentration is 5 $\mu\text{g/mL}$ (N=10–20). Which of the following is the most appropriate next step in management?

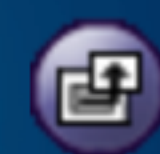
- ☐ A) Routine prenatal care
- ☐ B) Adding valproic acid to the medication regimen
- ☐ C) Increasing her current anticonvulsant medication
- ☐ D) Switching to carbamazepine
- ☐ E) Termination of the pregnancy



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

38. A 27-year-old primigravid woman at 30 weeks' gestation comes to the physician because of a 3-week history of pain in her arms and numbness in her hands that awaken her from sleep. She says that her hands feel as though she had been sleeping on them. Her pregnancy has been otherwise uncomplicated. Examination shows mild weakness of thumb abduction bilaterally. Sensation is decreased to pinprick in the index finger on the right. Which of the following is the most likely diagnosis?

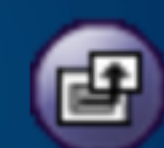
- ☐ A) Carpal tunnel syndrome
- ☐ B) Cervical radiculopathy
- ☐ C) Mononeuritis multiplex
- ☐ D) Reflex sympathetic dystrophy
- ☐ E) Thoracic outlet syndrome



Previous



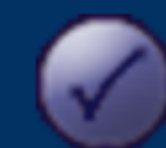
Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

39. A 14-year-old girl is brought to the physician because of painful episodes of menstrual cramping over the past 5 months; the pain has caused her to miss several days of school monthly. Menarche was at the age of 12 years. Her last menstrual period was 5 days ago. Examination shows no abnormalities. Which of the following is the most likely cause of her pain?

- ☐ A) Dermoid cyst
- ☐ B) Endometriosis
- ☐ C) Ovarian torsion
- ☐ D) Prostaglandin production
- ☐ E) Psychological factors



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

40. A 32-year-old woman, gravida 1, para 1, at 34 weeks' gestation is admitted to the hospital in labor. Pregnancy had been complicated by blurred vision and a 2.3-kg (5-lb) weight gain over the past week. Her last routine prenatal visit 1 month ago showed no abnormalities. Her temperature is 37°C (98.6°F), pulse is 80/min, respirations are 20/min, and blood pressure is 150/110 mm Hg. Examination shows retinal arteriolar spasms and pedal edema. Laboratory studies show:

Hemoglobin	12.5 g/dL
Leukocyte count	8000/mm ³
Platelet count	200,000/mm ³
Serum creatinine	1.2 mg/dL
Urine protein	2+

This patient's fetus is at greatest risk for which of the following obstetric complications?

- ☐ A) Brachial plexus injury
- ☐ B) Fetal growth restriction
- ☐ C) Hydrops
- ☐ D) Macrosomia
- ☐ E) Pulmonary hypoplasia
- ☐ F) Sepsis

41. A 52-year-old postmenopausal woman comes to the physician for a health maintenance examination. She does not smoke cigarettes or drink alcohol. Her diet is low in fat. She receives estrogen and progestin replacement therapy for vasomotor symptoms. Her blood pressure is 130/76 mm Hg. Examination shows no abnormalities. This patient is at greatest risk for which of the following types of cancer?

- ☐ A) Breast
- ☐ B) Cervical
- ☐ C) Endometrial
- ☐ D) Lung
- ☐ E) Ovarian



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

42. A 27-year-old primigravid woman at 37 weeks' gestation is admitted to the hospital in labor after an uncomplicated pregnancy. Fetal heart tones are reactive. Spontaneous rupture of membranes occurs with moderate blood-stained fluid, followed by a deep, persistent fetal heart bradycardia. The uterus is soft and nontender. The cervix is 3 cm dilated; there is a vertex presentation. Which of the following is the most likely diagnosis?

- | | |
|---|--|
| ☐ A) Abruptio placentae | ☐ F) Incomplete abortion |
| ☐ B) Cervical cancer | ☐ G) Placenta previa |
| ☐ C) Cervical polyp | ☐ H) Ruptured uterus |
| ☐ D) Ectopic pregnancy | ☐ I) Threatened abortion |
| ☐ E) Hydatidiform mole | ☐ J) Vasa previa |



43. Three months after an uncomplicated vaginal delivery of a newborn at term, a 24-year-old woman, gravida 2, para 2, comes to the physician because of a 1-month history of pain with sexual intercourse. She breast-feeds her infant, who recently was treated for thrush. She awakens to feed him every 3 hours during the night. Her menses have not resumed. Vital signs are within normal limits. Physical examination shows no abnormalities. Pelvic examination shows an erythematous vagina with no discharge. The cervix is closed, and the uterus is normal in size. Which of the following is the most appropriate next step in management?

- ☐ A) Recommendation for use of a lubricant
- ☐ B) Pap smear
- ☐ C) Vaginal culture
- ☐ D) Antibiotic therapy
- ☐ E) Antifungal therapy



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review

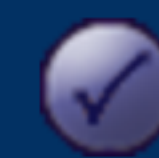
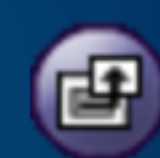


Help



Pause

44. A 21-year-old primigravid woman at 41 weeks' gestation is admitted to the hospital in labor. Her pregnancy has been uncomplicated. Contractions occur every 3 minutes. The cervix is 100% effaced and 4 cm dilated; the vertex is at +1 station. The membranes rupture yielding moderately thick meconium-stained fluid. The fetal heart rate has a baseline of 130/min with variable decelerations lasting 45 seconds and decreasing to 60/min. Which of the following is the most appropriate next step in management?
- ☐ A) External cephalic version
 - ☐ B) Forceps delivery
 - ☐ C) Amnioinfusion
 - ☐ D) Amniocentesis
 - ☐ E) Cordocentesis



The response options for the next 2 items are the same. Select one answer for each item in the set.

For each patient with abdominal or pelvic pain, select the most likely diagnosis.

- ☐ A) Adenomyosis

☐ B) Adnexal torsion

☐ C) Appendicitis

☐ D) Diverticulitis

☐ E) Ectopic pregnancy

☐ F) Endometriosis

☐ G) Endometritis

☐ H) Inflammatory bowel disease
- ☐ I) Leiomyomata uteri

☐ J) Ovarian carcinoma

☐ K) Pelvic inflammatory disease

☐ L) Primary dysmenorrhea

☐ M) Renal calculus

☐ N) Ruptured corpus luteum cyst

☐ O) Spontaneous abortion

45. A 28-year-old nulligravid woman has had increasingly severe dysmenorrhea over the past 2 years and dyspareunia and pain with defecation for 6 months. She has been unable to conceive for 2 years. Pelvic examination shows a fixed, tender, retroverted uterus, nodularity of the uterosacral ligaments, and tender adnexa bilaterally; there is a 5-cm, tender, cystic, right adnexal mass. A pregnancy test is negative.

For each patient with abdominal or pelvic pain, select the most likely diagnosis.

- | | |
|---|--|
| ☐ A) Adenomyosis | ☐ I) Leiomyomata uteri |
| ☐ B) Adnexal torsion | ☐ J) Ovarian carcinoma |
| ☐ C) Appendicitis | ☐ K) Pelvic inflammatory disease |
| ☐ D) Diverticulitis | ☐ L) Primary dysmenorrhea |
| ☐ E) Ectopic pregnancy | ☐ M) Renal calculus |
| ☐ F) Endometriosis | ☐ N) Ruptured corpus luteum cyst |
| ☐ G) Endometritis | ☐ O) Spontaneous abortion |
| ☐ H) Inflammatory bowel disease | |

46. A 27-year-old woman, gravida 3, para 3, had the sudden onset of severe, sharp pain in the right lower quadrant of the abdomen, pain in the right shoulder, light-headedness, nausea, and rectal pressure 6 hours ago. She uses a diaphragm for contraception, and her last menstrual period was 24 days ago. Her blood pressure is 120/70 mm Hg, and pulse is 80/min with no orthostatic changes. There is moderate tenderness of the right lower quadrant of the abdomen without guarding or rebound tenderness; bowel sounds are active. Culdocentesis shows 15 mL of nonclotting, serosanguineous fluid with a hematocrit of 5%. A pregnancy test is negative.



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

47. A previously healthy 37-year-old woman comes to the physician because of a 5-week history of bloody nipple discharge from the left breast. Examination of the breasts shows no palpable masses or skin retraction. Blood can be expressed from a ductal element of the nipple of the left breast. Mammography shows no abnormalities. Which of the following is the most likely diagnosis?

- ☐ A) Fat necrosis
- ☐ B) Fibroadenoma
- ☐ C) Fibrocystic changes of the breast
- ☐ D) Galactorrhea
- ☐ E) Infiltrating ductal carcinoma
- ☐ F) Intraductal papilloma
- ☐ G) Mastitis
- ☐ H) Paget disease of the breast





48. A 22-year-old primigravid woman at 39 weeks' gestation has had ruptured membranes for 5 hours without contractions. Her prenatal course was uncomplicated. Her cervix is 80% effaced and 2 cm dilated. The fetal position is right occipitoposterior. The fetal heart rate pattern is shown. Which of the following is the most likely explanation for this pattern?
- ☐ A) Chorioamnionitis
 - ☐ B) Fetal sleep state
 - ☐ C) Occipitoposterior position
 - ☐ D) Umbilical cord compression
 - ☐ E) Uteroplacental insufficiency

49. A 42-year-old woman, gravida 5, para 5, comes to the physician requesting advice regarding contraception. Menses occur at regular 28-day intervals. Her last menstrual period was 3 days ago. She has hypertension treated with a β -adrenergic blocking agent, but she often forgets to take it. At the age of 21 years, she was treated for *Chlamydia trachomatis* infection. During her second pregnancy, a Pap smear was reported as a low-grade squamous intraepithelial lesion; after delivery, a follow-up Pap smear showed no abnormalities. Since that time, she has had no abnormal Pap smears. She is sexually active and monogamous with her husband. Physical examination, including pelvic examination, shows no abnormalities. She desires the most effective contraception that she will easily remember to use. Which of the following is the most appropriate recommendation regarding contraception for this patient?

- ☐ A) Condoms
- ☐ B) Oral contraceptive
- ☐ C) Diaphragm
- ☐ D) Depot medroxyprogesterone
- ☐ E) Placement of an IUD



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

50. Two days after a cesarean delivery for fetal distress, a 27-year-old woman has temperatures to 38.2°C (100.8°F). She had an 8-hour labor complicated by mild meconium-stained amniotic fluid. Membranes ruptured 2 hours after admission. Breath sounds are decreased at both lung bases. Abdominal examination shows mild tenderness below the umbilicus and a well-healing incision. There is a moderate amount of lochia with no odor. Examination of the lower extremities shows edema and no tenderness. Her hemoglobin concentration is 10.8 g/dL, leukocyte count is 14,000/mm³, and platelet count is 189,000/mm³. A urine catheter specimen is negative. Which of the following is the most likely diagnosis?

- | | |
|---|---|
| ☐ A) Atelectasis | ☐ G) Pulmonary embolism |
| ☐ B) Deep venous thrombosis | ☐ H) Pyelonephritis |
| ☐ C) Endometritis | ☐ I) Septic pelvic thrombophlebitis |
| ☐ D) Episiotomy infection | ☐ J) Ureteral obstruction |
| ☐ E) Mastitis | ☐ K) Urinary tract infection |
| ☐ F) Pneumonia | |

